



Innovation & Differentiation Note

What is new about Medreach, and why it is different from maps, bed portals and helplines.

☆ MPOne Idea & Innovation Hackathon 2026 🏠 Challenge 5 · AI for Public Services · Healthcare

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1 Medreach compared with what exists today

Capability	Map search for "nearest hospital"	Static bed / hospital portals	Phone helplines	Medreach
Understands the emergency (voice, Hindi)	No	No	Operator	AI, voice, Hinglish
Matches required capability (specialist on duty, equipment working)	No	Beds only	Manual	Yes, deterministic
Shows freshness & who updated data	No	Rarely	No	Every figure
Hospital accepts before travel	No	No	Sometimes	Yes + bay
Automatic fallback to next hospital	No	No	Manual	60 s timeout
Pre-arrival note for ER doctor	No	No	No	AI SBAR note
Live map for citizen AND hospital	Citizen only	No	No	Both, live
Help for a person alone	No	No	No	"I'm alone" mode
Works offline / weak network	Partly	No	Voice call	Offline PWA

2 Ten innovations in Medreach



1. AI → requirements, not diagnosis

AI converts messy words into structured needs (type, severity, red flags, age group, capabilities). It never names a disease as fact: "may indicate..."



2. Capability-first matching

A hospital qualifies only if the service exists, the specialist is on duty and the machine works now; then time to care decides the order.



3. Data freshness as a first-class signal

Green / amber / red verification age on every number, weighted into the score. Stale data can never look confirmed.



4. Three truths kept separate

Reported capacity ≠ referral accepted ≠ confirmed destination. The citizen always knows exactly how certain things are.



5. Safety floor for AI

Severity = max(AI, rules). AI can raise urgency but never lower it; if AI is offline the rules engine still works.



6. Explainable score

"Why this hospital?" checklist first, score second. Tap the score to see time to care, bed-on-arrival × freshness, services, facility level.



7. "I'm alone" mode

One tap: location shared → trusted contact alerted with tracking link → hospital request → acceptance → ambulance, with a live checklist.



8. AI pre-arrival SBAR note

The ER receives Situation, Background, Assessment, Recommendation before the patient arrives, so the bay and specialist are ready.



9. Live maps on both sides

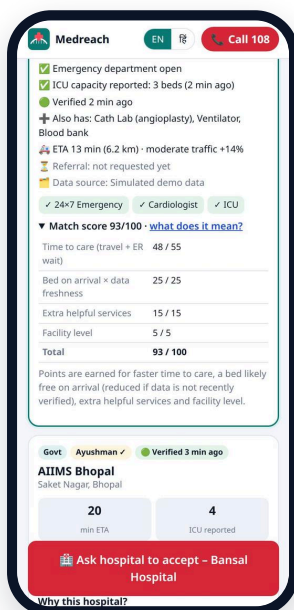
Citizens see hospitals by ER status and their ambulance moving; hospitals see incoming patients and ETAs, without API keys (free OpenStreetMap).



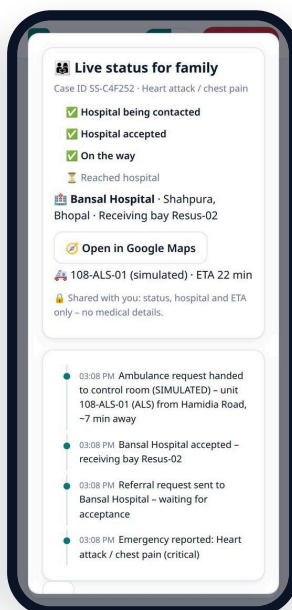
10. Offline-first, same brain everywhere

Triage and matching code runs on the server AND on the phone, so an emergency is still understood without internet.

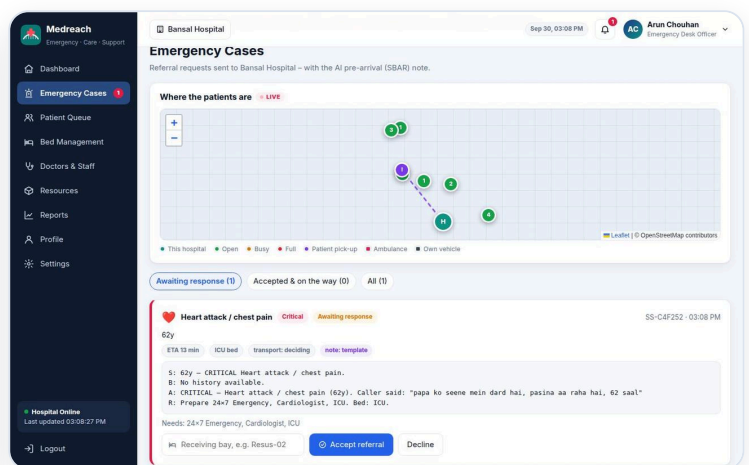
3 Innovation you can see in the product



"Why this hospital?"
checklist + explained score



Family link: status, hospital,
ETA, no medical data



Hospital sees the AI SBAR note, masked identity and accepts with a receiving bay

Live proof of the architecture: when a hospital officer sets the ER to *Diverting*, that hospital disappears from every citizen's list within a second. Re-open it and it returns as "Verified just now".

4 Why this is hard to copy

- Workflow, not just data: the value comes from joining citizen, AI, hospital staff and transport in one closed loop with acceptance.
- Trust model: staff-only updates with OTP login, roles, audit log and freshness make the numbers believable for government use.
- Built for Bharat: Hindi voice, Hinglish understanding, Ayushman / ABHA awareness, 108 always one tap away, low-bandwidth PWA.
- Privacy by design: consent, masking, hospital-scoped access and 24 h retention are built in, aligned with the DPDP Act 2023.